

العمر(تاريخ الميلاد) : 07-03-1981 المهنة : ربه منزل

رئيس
المجلس الطبي المتخصص

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Name: جوفانه عادل مورييس شحاته National ID: 28103070102608	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment
--

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية

بطاقة تعريف الشخصية

جوفانه
عادل مويرس شحاته
هـ ش احمد السيد-المعصرة البلد
المعصرة - القاهرة

٢٨١٠٣٠٧٠١٠٢٦٠٨



LV4381466





٢٠٢٦/٠٧ ٢٨١٠٣٠٧٠١٠٢٦٠٨

حاصله على بكالوريوس علوم وتربية

رياض أطفال

متزوجة مسيحية

شريف رمزي اسكندر ابراهيم

البطاقة سارية حتى ٢٠٣٣/٠٧/٠٤



MRN:	10126283088	D.O.B:	07/03/1981
Patient Name:	Gouvana ADEL MOREES SHEHATA	Sex:	Female
IC No.:	28103070102608		
Location:	External Clinic		

SURGICAL PATHOLOGY

Biopsy Small - (1 Site)

Specimen Id	2607210100	Collection Date	21/07/2026 10:14 AM
Accession No	SUR-26-2554	Received Date	21/07/2026 10:14 AM
Sample Type	Tissue	Doctor	shrouk mahmoud mohamed Mohamed

Clinical Information

Right breast lesion at 9-10 O'clock (BIRADS 5).

Gross Picture

Multiple grayish tissue cores 0.6-1.3 cm long, totally submitted.

Microscopic Picture

A case of invasive duct carcinoma, grade 3. No DCIS component. There is evidence of lymphovascular emboli. Stroma is desmoplastic with mild inflammatory cell reaction.

Diagnostic Summary

Tumour type:

Invasive duct carcinoma, NST, grade 3.

Laterality

Right

Specimen type:

U/S guided core biopsy.

Location

9-10 O'clock

Lesion Size

Invasive tumor size/s (mm):

Total extent of lesion (mm):

Multiple Lesions

Histological grade

Mitosis score (1-3):

3

Tubular formation score (1-3):

3

Nuclear grade (1-3):

3

Histological grade (1-3):

3

Peritumoural lymphovascular invasion

Present

DCIS in specimen

Absent

Diagnosis

Right breast lesion, U/S guided core biopsy:

Invasive duct carcinoma, NST, grade 3.

Comments

ER, PR & HER-2 are available(upon request).

Biopsy reporting category

B5b= Invasive cancer

Signature

Dr. Ahmed Ali. MD.

Co-signed by:

Dr. Hazem Abdelazim, MD, IFCAP

Reported and electronically endorsed by

Hazem Abdulazim Sobhy Abo Ismael

Pathology lab director:

Dr. Hazem Abdelazim, MD, IFCAP

Laterality

Signature

Dr.Mahitab Eltohamy ,MD

Co-Signed by:

Reported and electronically endorsed by

Mahitab Mohamed Eltohamy Mohamed

Pathology lab director:

Dr. Hazem Abdelazim, MD, IFCAP

Validated on 09/08/2026 04:56 PM

Print Date:

11/08/2026 11:52 AM

Page Number: 2 of 2



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مركز بهية الهرم
4 شارع علوية أول شارع الهرم بجوار داري , الجيزة

مستشفى بهية الشيخ زايد
٤ - الحي الأول - الشيخ زايد - كلية الهندسة

[[DOCTYPE:PATHOLOGY_REPORT]]

MRN: 10126283088
Patient Name: Gouvana ADEL MOREES SHEHATA
IC No.: 28103070102608
Location: External Clinic

D.O.B: 07/03/1981
Sex: Female

IMMUNOPATHOLOGY

Her2 Receptors

Specimen Id 2608020098
Accession No IMM-26-3190
Sample Type Paraffin Block

Collection Date 02/08/2026 10:56 AM
Received Date 02/08/2026 10:56 AM
Doctor Mahmoud sayed Abdelmohsen Meshref

Immunohistochemical Report

pathology number
2554/26

Clinical Data
Rt. breast cancer

Gross Picture :
Referred block submitted for HER 2 immunostaining.

System Used :
Ventana BenchMark ULTRA autostainer.

Antibody:
Ventana anti-HER-2 [4B5] rabbit monoclonal antibody.

Scoring system :
Reference: ASCO-CAP HER2 Test Guideline Recommendations, 2018. IHC 0 :as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within $\leq 10\%$ of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within $>10\%$ of the invasive tumor cells. IHC 2+: The revised definition of IHC 2+ (equivocal) is weak to moderate complete membrane staining observed in $>10\%$ of tumor cells.. IHC 3+: based on circumferential membrane staining that is complete, intense in $>10\%$ of tumor cells.

(Laterality) breast Cancer:
Right

Her2 Result
Equivocal [score 2+]. Recommended For SISH

Comment :
**SISH test for HER-2 gene copy number is available on request.



MRN: 10126283088
 Patient Name: Gouvana ADEL MOREES SHEHATA
 IC No.: 28103070102608
 Location: External Clinic
 D.O.B: 07/03/1981
 Sex: Female

IMMUNOPATHOLOGY

Hormone receptors

Specimen Id 2608020096
Accession No IMM-26-3191
Sample Type Paraffin Block

Collection Date 02/08/2026 10:56 AM
Received Date 02/08/2026 10:56 AM
Doctor Mahmoud sayed Abdelmohsen Meshref

Immunohistochemical Report

pathology number
 2554/26

Clinical Data
 Rt. breast cancer

Gross Picture :
 Referred block submitted for ER/PR immunostaining.

System Used :
 Ventana BenchMark ULTRA autostainer.

Antibody:
 Ventana anti-PR[1E2] Rabbit monoclonal antibody, Ventana anti-ER[SP1] Rabbit monoclonal antibody.

Scoring system :
 According to the ASCO/CAP guideline update, 2019 : Samples with 1% to 100% of tumor nuclei positive for ER or progesterone receptor (PgR) are interpreted as positive. If ER (not PgR), 1% to 10% of tumor cell nuclei are immunoreactive, the sample are reported as ER Low Positive. A sample is considered negative for ER or PgR if < 1% or 0% of tumor cell nuclei are immunoreactive.

Allred score: proportion of cells staining (0, no staining; 1, <1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak, 2, intermediate; and 3, strong). Total score from 0 to 8.

Comment:
 - There are much more limited data on the 2% to 3% of cancers that are low ER expressers (most often defined as 1%-10% ER-positive cells by IHC) and their potential benefit from endocrine therapy.



MRN: 10126283088
Patient Name: Gouvana ADEL MOREES SHEHATA
IC No.: 28103070102608
Location: External Clinic

D.O.B: 07/03/1981
Sex: Female

SURGICAL PATHOLOGY

Biopsy Small - (1 Site)

Specimen Id 2607210100
Accession No SUR-26-2554
Sample Type Tissue

Collection Date 21/07/2026 10:14 AM
Received Date 21/07/2026 10:14 AM
Doctor shrouk mahmoud mohamed Mohamed

Clinical Information

Right breast lesion at 9-10 O'clock (BIRADS 5).

Gross Picture

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Microscopic Picture

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Tumour type:

Invasive duct carcinoma, NST, grade 3.

Laterality

Right

Specimen type:

U/S guided core biopsy.

Location

9-10 O'clock

Lesion Size

Invasive tumor size/s (mm):

Total extent of lesion (mm):

Multiple Lesions

Histological grade

Mitosis score (1-3):

3



- PR positive in 1-20% of tumor cells is considered as Low positive.

(Laterality) breast Cancer:

Right

Diagnosis :

ER : positive, strong nuclear reaction in 99 % of tumor cells (8/8).

PR :positive, strong nuclear reaction in 100% of tumor cells (8/8).

Signature

Dr.Mahitab Eltohamy ,MD

Reported and electronically endorsed by

Mahitab Mohamed Eltohamy Mohamed

Pathology lab director:

Dr. Hazem Abdelazim, MD, IFCAP

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مركز بهية الهرم
4 شارع علوية أول شارع الهرم بجوار داري , الجيزة

مستشفى بهية الشيخ زايد
٤٠ الحي الأول - الشيخ زايد، القاهرة، الهندسة

[[DOCTYPE:PATHOLOGY REPORT]]

Tubular formation score (1-3):

3

Nuclear grade (1-3):

3

Histological grade (1-3):

3

Peritumoural lymphovascular invasion

Present

DCIS in specimen

Absent

Diagnosis

Right breast lesion, U/S guided core biopsy:

Invasive duct carcinoma, NST, grade 3.

Comments

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Biopsy reporting category

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Co-signed by:

Dr. Hazem Abdelazim, MD, IFCAP

Reported and electronically endorsed by

Hazem Abdulazim Sobhy Abo Ismael

Pathology lab director:

Dr. Hazem Abdelazim, MD, IFCAP

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مركز بهية الهرم
4 شارع علوبة أول شارع الهرم بجوار نادي الجيزة

مستشفى بهية الشيخ زايد
4. الحي الأول - الشيخ زايد لأكاديمية الهندسة

[[DOCTYPE:PATHOLOGY REPORT]]

PATIENT NAME: Gouvana ADEL MOREES SHEHATA

GENDER: Female

DATE : 20-Jul-2026 12:39 PM

PATIENT ID: 10126283088

AGE: 45 y

ORDER: 3963046,3963047

**FULL FIELD DIGITAL MAMMOGRAPHY AND COMPLEMENTARY
ULTRASOUND****INDICATION:**

- Right breast palpable lump since two months.

TECHNIQUE:

- CC and MLO views.
- Complementary ultrasound.

REPORT:

- Heterogenous dense breasts (ACR C).

Right Breast:

- UOQ two ill defined spiculated dense lesions measuring roughly about 4.4x4.5 cm, 3 cm away from the nipple (showing internal pleomorphic calcifications) & 3x4.1 cm, 4 cm away from the nipple. (measured in CC view)
- Complementary US revealed UOQ multiple ill defined spiculated hypoechoic lesions are seen at:
 - 11-1:00 measuring about 2x3.6 cm, 0.9 cm away from the overlying skin. Prominent ducts are seen related to the medial aspect of the lesion, ? intraductal extension.
 - 9-10:00 measuring 2x4.7 cm, 0.7 cm away from the overlying skin.
 - Two small satellite lesions are seen at 11:00 & 10:00 measuring about 0.9x1.2 cm (0.9 cm away from 10:00 index lesion) & 0.5x0.6 cm (0.3 cm away from 12:00 index lesion) respectively.
- Normal skin thickness and contour.
- Normal nipple areola complex.
- No detected pathological axillary or supraclavicular lymph nodes.

Left Breast:

- No spiculated lesions or grouped microcalcifications.
- Complementary Ultrasound showed no detected solid or cystic lesions.
- Normal skin thickness and contour.
- Normal nipple areola complex.

PATIENT: Gouvana ADEL MOREES SHEHATA, 10126283088, Female, 45 y

Report version 1, 20-Jul-2026 02:11 PM

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مركز بهية الهرم
1 شارع طلوية اول شارع الهرم بجوار داري ، الجزيرةمستشفى بهية الشيخ زايد
في الجزيرة الشيخ زايد امام كلية الهندسة

[DOCTYPIC MAMMOGRAM]

- Nonspecific axillary lymph nodes.

OPINION:

- Right breast UOQ multiple suspicious lesions, for histopathological assessment (BIRADS 5).
- Left breast adenosis (BIRADS 2).

RECOMMENDATION:

Referral to clinic

Reported By,

Dr. Lamiaa Abdallah, MSc

20-Jul-2026 02:11 PM

PATIENT: Gouvana ADEL MOREES SHEHATA, 10126283088, Female, 45 y
Report version 1, 20-Jul-2026 02:11 PM

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مركز هيئة الهرم
4 شارع طلوعة أول شارع الهرم بجوار دارى . الجيزة

مستشفى هيئة الشيخ زايد
الشيخ زايد امام كلية الهندسة



[[DOCTYPE:MAMMOGRAM]]

Patient's Name: Jovanna Adel Mouris Shehata
Date: 2026-09-02
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

MRI OF THE BRAIN WITH CONTRAST :

Axial T1, T2 ,FLAIR,
Coronal T2
Sagittal T1
Postcontrast T1

Findings:

- Few tiny bilateral cerebral periventricular foci of abnormal MR signal are seen displaying low signal on T1 that turned bright on T2 & FLAIR with no contrast uptake. No mass effect or perifocal oedema.
- Normal size and configuration of the ventricular system. Bright T2 & FLAIR signal are seen framing the periventricular white matter.
- No midline shift or deformity
- Prominent cortical sulci and extra axial CSF spaces.
- No extra axial collections or intra axial masses.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.
- Prominent peri-optic CSF Spaces bilaterally.

OPINION:

*Few tiny bilateral cerebral lacunar infarctions.
Diffuse subcortical arteriolo-sclerotic leukoencephalopathy.
Prominent peri-optic CSF spaces bilaterally for clinical correlation.*

Dr.sherif Wagdy